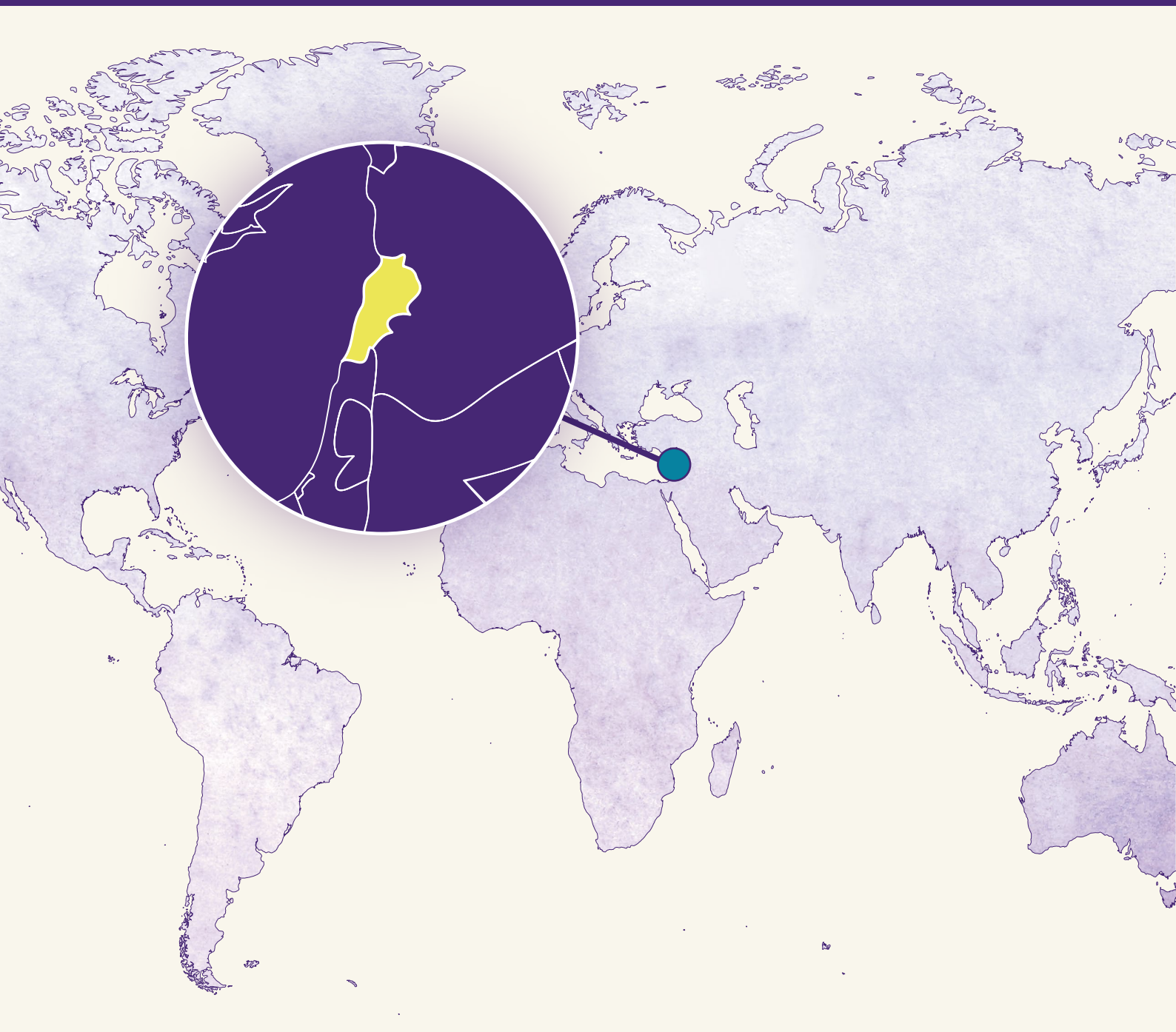


LEBANON

Violence Against Health Care in Conflict 2025



**SAFEGUARDING
HEALTH
IN CONFLICT**



**Insecurity
Insight**
Data on People in Danger



REPORTED INCIDENTS AND MOST COMMONLY REPORTED CONCERNS

 REPORTED INCIDENTS	 INCIDENTS AFFECTING EMERGENCY MEDICAL SERVICES	 INCIDENTS WHERE HEALTH FACILITIES WERE DAMAGED/ DESTROYED	 HEALTH WORKERS KILLED
2025			
15	9	6	3
2024			
497	310	209	408
2023			
18	9	2	9

Source: Insecurity Insight for the SHCC.

OVERVIEW

Using data collected by Insecurity Insight, the Safeguarding Health in Conflict Coalition (SHCC) identified 15 incidents of violence against or obstruction of health care in Lebanon in 2025, compared to 497 in 2024 and 18 in 2023.¹ Among incidents reported in 2025, emergency medical services (EMS) were attacked on at least nine occasions and health facilities were damaged at least six times. At least three health workers were killed.

Methodology and sources

Information on incidents of violence against health care in Lebanon is compiled from open and private sources and information projects. Data is continuously updated and numbers may change if new information is made publicly available. For the latest available figures, see Insecurity Insight's [global map](#) on attacks on health care. See [Methodology](#) for further information.

THE CONTEXT

In 2025, Lebanon remained affected by a non-international armed conflict between Israel and the non-state armed group Hezbollah. Despite a [ceasefire](#) announced on November 27, 2024, [violations](#) continued throughout 2025, resulting in civilian deaths and injuries and the widespread damaging or destruction of essential infrastructure.



Ongoing violence hindered reconstruction efforts, with Lebanon's recovery and reconstruction needs estimated at USD 11 billion by March 2025. By 2025, approximately 4.1 million people remained in need of humanitarian assistance, with thousands facing high levels of acute food insecurity, including around 22,000 people in IPC Phase 4 ("Emergency") between November 2025 and March 2026. By September 2025, around 82,000 people remained displaced due to ongoing Israel Defense Forces (IDF) armed violence and its occupation of parts of southern Lebanon, and by October approximately 64,000 were still internally displaced, primarily from southern border areas, including Bint Jbeil, Marjayoun, and El Nabatieh.

The early 2025 United States Agency for International Development (USAID) aid freeze disrupted planned US funding flows and contributed to the reduced availability of humanitarian assistance in Lebanon, particularly for nutrition, food security, and refugee support programs.

VIOLENCE AGAINST OR OBSTRUCTION OF HEALTH CARE IN 2025

In 2025, violence against or obstruction of health care decreased dramatically in Lebanon following the ceasefire in late 2024, but overall insecurity persisted. Incidents were reported in three governorates (three fewer than the previous year), with most cases continuing to occur in Bint Jbeil, Marjayoun and Nabatieh districts in Lebanon's southern Nabatieh governorate.

Over half of the reported incidents involved attacks on EMS during the first five months of 2025. Damage to health facilities and health worker killings continued, despite the ceasefire, although at lower levels than in 2024. Maternal care services in Lebanon were disrupted when nearby IDF air strikes interrupted routine health services at a school, causing panic among children and teachers.²

All but one of the reported incidents were attributed to the IDF, often involving drone- or aircraft-delivered strikes that damaged health facilities and ambulances or struck areas in the vicinity of hospitals. In two cases, the IDF used drones to drop stun grenades on a health center and sound bombs on emergency workers as part of "double-tap" strikes while these workers were conducting recovery and rescue efforts after an earlier strike.³ The IDF also used sniper fire and live gunfire against emergency responders during recovery operations.

One incident was not attributed to the IDF: gunmen opened fire inside a hospital in Bekaa governorate, which was reportedly linked to a dispute involving a group member who had recently been admitted to the hospital.⁴

A similar number of cases affected health care providers working for national health structures and providers affiliated with Hezbollah or the Amal Movement. NGOs, private health care organizations, Red Cross societies and UN agencies were also affected by IDF attacks.

Attacks on emergency medical services

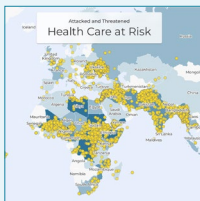
EMS were directly affected by violence at least nine times between January and May 2025, compared to 310 times in 2024. Three emergency workers were killed and two injured, with most incidents occurring in Bint Jbeil, Marjayoun, and Nabatieh districts in Nabatieh governorate. Along with the previously mentioned drone-launched stun grenades and sound bombs, emergency workers were killed and injured by IDF armed drone and aircraft strikes (which in some instances were reported as being part of "double-tap" strikes) or sniper fire while conducting body retrievals or responding to emergencies. Two paramedics



were killed by IDF gunfire as Lebanese families returned to their homes in southern Lebanon following a period of reduced hostilities.⁵

Health facilities damaged

In 2025, health facilities were damaged at least six times, compared to 209 in 2024. In addition to the previously mentioned hospital attack by gunmen, four health facilities, including a newly established health center and a medical storage facility, were destroyed or severely damaged by IDF air strikes, while a fifth facility was struck by a stun grenade dropped from an IDF drone.⁶



This chapter is based on 2023-2025 LBN SHCC Health Care Data. Download this data on the [Humanitarian Data Exchange \(HDX\)](#).

Insecurity Insight updates data continuously and numbers may change if new information is made publicly available. For the latest available figures, see Insecurity Insight's [global map](#) on attacks on health care.

THE IMPACT OF ATTACKS ON HEALTH CARE

With hostilities in Lebanon still ongoing despite the ceasefire, the reduced capacity to medically evacuate wounded and sick and the damaging or destruction of health facilities represented a loss of care capacity for those in need that significantly affected the survival chances of people living in areas where shelling and bombing occurred. Moreover, the losses represented by both the killing of health care professionals and the damaging or destruction of medical infrastructure are unlikely to be reinstated in the most affected zones, creating long-lasting gaps in the health care system.

Years of prolonged conflict and violence in Lebanon have led to widespread mental health challenges and psychological trauma. Children have suffered severe physical and mental impacts from the conflict. There is an urgent need for mental health and psychosocial support, while persistent social stigma continues to impede child protection and mental health interventions. In February 2025, UNICEF reported that 45% of households had to reduce health spending and 33% lacked essential medications for their children. Médecins Sans Frontières (MSF) patients described the ongoing trauma experienced in particular by children, with one noting that their daughter “faints at the sound of any strike, even if it’s far away,” underscoring the long-term consequences of attacks on health care and the urgent need for protective and psychosocial support.

Health needs that emerged during the conflict were further exacerbated by displacement, which restricted access to health care, placed additional strain on already overburdened health systems, limited the availability of essential medicines, and caused additional physical and psychological harm. Displacement likely worsened outcomes for individuals with non-communicable diseases in 2025, because people’s attempts to escape from conflict-affected areas made it difficult for them to access essential medications. The damaging or destruction of Lebanon’s water supply infrastructure, compounded by drought, insecurity, and attacks on health care, continued to heighten the risk of waterborne diseases such as cholera, hepatitis A, and rotavirus, underscoring the urgent need for resilient water supply and health care systems.



Health needs were further impacted by food insecurity. Conflict and displacement drove high rates of malnutrition by limiting access to agricultural imports, arable land, skilled labor, and safe markets, disproportionately impacting agriculture-dependent governorates, degrading the food system, and heightening food insecurity. The destruction of water and food supply system infrastructure, compounded by the 2025 drought, drove high levels of food insecurity. Qualitative interviews by Action Against Hunger (ACF), Insecurity Insight, and Oxfam underscored the extensive, long-term impact of armed violence on communities and civilian infrastructure.

The functioning of health care infrastructure grew worse during 2025. By March, 33 primary health care centers and eight hospitals were completely closed. In addition to weakened and over-burdened health infrastructure, issues like violence, injury, trauma, and displacement all served as barriers to health workers being able to deliver care and carry out their roles. In December 2025, the International Organization for Migration, Lebanese Ministry of Public Health and International Lebanese Medical Association launched a project called Connecting Global Talent: Diaspora Engagement in Healthcare-Phase II that was designed to link Lebanon's medically trained diaspora with local health care professionals.

- 1 The SHCC's 2024 Annual Report cited 485 incidents of violence against or obstruction of health care in Lebanon in 2024, 18 in 2023. Data is continuously updated and numbers can change if new information is made publicly available.
- 2 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 LBN SHCC Health Care Data. Incident number 100256.
- 3 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 LBN SHCC Health Care Data. Incident numbers 120878; 91152.
- 4 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 LBN SHCC Health Care Data. Incident number 101571.
- 5 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 LBN SHCC Health Care Data. Incident numbers 91149; 105791.
- 6 Insecurity Insight. Safeguarding Health in Conflict Coalition 2025 Report Dataset: 2023-2025 LBN SHCC Health Care Data. Incident numbers 96303; 102004; 120878.

SAFEGUARDING HEALTH IN CONFLICT

The Safeguarding Health in Conflict Coalition is a group of more than 40 organizations working to protect health workers and services threatened by war or civil unrest. We have raised awareness of global attacks on health and pressed United Nations agencies for greater global action to protect the security of health care. We monitor attacks, strengthen universal norms of respect for the right to health, and demand accountability for perpetrators. Suggested citation: Safeguarding Health in Conflict Coalition (SHCC) 2026. Care in the Crosshairs: Violence Against Health Care in Conflict, May 2026. <https://shcc.pub/SHCCAnnualReport2025>
<https://safeguarding-health.com>

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